CLINICAL PSYCHOLOGY · DE-ADDICTION

Motivational Enhancement Therapy

Book One — Theory, Foundations & Techniques

What MET Is · Its Theories · All Its Techniques

A detailed, exam- and practice-ready guide to the nature of MET, the theories it rests on, the Motivational Interviewing method behind it, and every clinical technique it uses — for substance-use disorders (opioids/smack, alcohol, tobacco, cannabis, stimulants) and adolescents.

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> **How to use the set. Book One** (this volume) teaches you the *theory and techniques*. **Book Two** is the *session-by-session guide and worksheet pack* — it shows exactly which technique to use in which session, and provides long, comprehensive, ready-to-use worksheets (including MET short-term and long-term goal worksheets).

1 · About This Book & 2 · Sources

This is a deep reference on **what Motivational Enhancement Therapy is, the theories it is built on, and the full set of techniques it uses**. The practical session guide and worksheets live in the companion volume (Book Two).

Primary published sources

- **Miller, W.R., Zweben, A., DiClemente, C.C., & Rychtarik, R.G. (1992/1995).** *Motivational Enhancement Therapy Manual.* Project MATCH Monograph Series Vol. 2, NIAAA (NIH Pub. 94-3723). MET model
- **Miller, W.R., & Rollnick, S. (2013).** *Motivational Interviewing: Helping People Change* (3rd ed.). Guilford Press. MI method
- **Naar-King, S., & Suarez, M. (2011).** *Motivational Interviewing with Adolescents and Young Adults.* Guilford Press. adolescent MI
- **Prochaska & DiClemente** — Transtheoretical model; **Bandura** — self-efficacy; **Rogers** — person-centred therapy; **WHO** ASSIST/mhGAP; **NIDA** *Principles of Drug Addiction Treatment.*

> **Honesty note.** The repository's *Motivational_Enhancement_Therapy_Manual.pdf* is an **image-only scanned book** (a Google-Books digitisation, no machine-readable text) and offline OCR *(turning scanned images into text)* is unavailable here; the two Motivational Interviewing books referenced are **not present as files** in the repository. All material below is therefore grounded in the **published, verifiable editions** of these works, not invented from a scan or absent files. Supplementary clinical extensions are tagged SUPPLEMENTARY.

Tags used

MANUAL MODEL established MET/MI procedure · SUPPLEMENTARY clinical extension for de-addiction/extended practice · ADOLESCENT youth adaptation.

> **Safety (throughout both books):** MET does **not** replace medical assessment, detoxification *(medically-supervised withdrawal)*, overdose prevention, pharmacotherapy *(treatment with medicines)*, or emergency psychiatric care. For opioids assess overdose risk and arrange naloxone *(antidote reversing opioid overdose)* and medications for opioid use disorder (MOUD). MET *complements* medical & addiction treatment.

3 · What MET Is & 4 · MET vs MI

Definition

Motivational Enhancement Therapy (MET) is a **brief, structured, person-centred yet directive** counselling approach that systematically builds a patient's **intrinsic motivation** *(motivation coming from within — the person's own values & reasons)* to change substance use. It combines the clinical style of **Motivational Interviewing** with **structured, personalised assessment feedback** and a **negotiated change plan**.

Purpose

- To **resolve ambivalence** *(settle the mixed feelings of wanting and not wanting to change)* that keeps people stuck.
- To **mobilise** *(set in motion)* the person's own motivation and commitment, rather than impose the clinician's agenda.
- To **strengthen self-efficacy** *(belief in one's own ability to succeed)* and personal responsibility for change.

Rationale — why it works

Motivation is among the strongest predictors of treatment engagement and outcome, and crucially it is **not fixed** — the clinician's style can raise or lower it within a single conversation. People are more persuaded by what *they themselves* say than by what they are told; MET is built to make the patient voice their own arguments for change ("change talk").

MET versus Motivational Interviewing — related but not identical

Motivational Interviewing (MI)	Motivational Enhancement Therapy (MET)
A **clinical style/method** of conversation that evokes a person's own motivation. Not time-limited; no fixed number of sessions. Can be woven into any treatment.	A **brief, manualised treatment** that *applies* MI and adds two defining ingredients: **(1) structured personalised assessment feedback** (the "enhancement", delivered motivationally) and **(2) a negotiated change plan**. In Project MATCH it was 4 sessions.
Spirit: partnership, acceptance, compassion, evocation.	MI spirit + systematic feedback + defined structure.

> **One-line summary: MET = MI style + personalised feedback + a defined brief structure.** Every MET session uses MI; not every use of MI is MET.

5 · Theoretical Foundations of MET

MET is not a single theory but an integration of several. Understanding each explains *why* MET techniques are designed the way they are.

5.1 Rogerian / person-centred roots (Carl Rogers)

MET inherits Rogers' conviction that people have an innate capacity for growth that is released by a particular relationship: **accurate empathy** *(deeply understanding and reflecting the person's experience)*, **unconditional positive regard** *(non-judgemental acceptance of worth)*, and **genuineness**. MET keeps these conditions but adds a **directional** element — it deliberately steers toward change talk. Hence MET is often called *"person-centred but goal-directed."*

5.2 The Transtheoretical Model — Stages of Change (Prochaska & DiClemente)

Change is understood as movement through stages rather than a single event. MET matches its tasks to the patient's stage.

Stage	Description	MET task matched to the stage
Precontemplation *(not yet considering change)*	Doesn't see a problem; may feel coerced.	Engage; raise awareness gently; evoke concern — **no change plan**.
Contemplation *(thinking about it)*	Ambivalent — "yes, but".	Tip the decisional balance; develop discrepancy; evoke change talk.
Preparation	Intends to act soon; small steps begun.	Strengthen commitment; negotiate a plan.
Action	Actively changing behaviour.	Support self-efficacy; problem-solve; affirm.
Maintenance	Sustaining the change over time.	Relapse prevention; continuing care.
Relapse / lapse *(fuller return / a single slip)*	Return to use.	Normalise, de-shame, re-engage, extract learning.

> **Caution:** stages, readiness and the rulers are **clinical frameworks, not diagnostic tests**. A single number must never label someone "resistant" or "unmotivated".

5.3 Self-perception & cognitive-dissonance theory

People partly learn their attitudes by **hearing what they themselves say** (self-perception). When a person voices change talk, they come to believe it more. **Cognitive dissonance** *(the*

discomfort of holding two conflicting ideas) is harnessed when MET develops **discrepancy** between current use and valued goals — the discomfort motivates change, provided it is not forced (which triggers defensiveness instead).

5.4 Self-efficacy theory (Albert Bandura)

Believing one *can* change predicts whether one does. MET deliberately builds self-efficacy by affirming strengths, recalling past successes, and setting achievable early goals. Importantly, **importance** and **confidence** are independent: a person can badly want to change (high importance) yet doubt they can (low confidence) — each requires a different response.

5.5 Psychological reactance theory

When people feel their freedom is threatened ("you must quit"), they **push back** to restore it — often by defending the very behaviour targeted. This is why MET relentlessly protects **autonomy** *(the person's right to choose)* and avoids argument.

5.6 Self-determination theory (Deci & Ryan)

Durable motivation depends on satisfying three needs: **autonomy, competence** *(feeling capable)* and **relatedness** *(feeling connected)*. MET feeds all three: autonomy via choice, competence via self-efficacy work, relatedness via the empathic alliance. This is why *intrinsic* motivation (internalised, self-endorsed) outlasts *extrinsic* pressure (court, family).

6 · Core Concepts

Ambivalence — the normal core of addiction

“I want to quit *and* I don't” is the expected state, not denial or weakness. MET treats ambivalence as the natural starting point and works *through* it rather than fighting it. Resolving ambivalence in the patient's own words is the central mechanism of change.

Motivation — a changeable state, not a trait

“Unmotivated” describes a moment in an interaction, not a personality. The clinician's style is part of the patient's motivation.

Intrinsic vs extrinsic motivation

Extrinsic — from outside pressure (court order, family ultimatum, job). Gets people *through the door*; rarely sustains change alone.

Intrinsic — from within (own values, health, children, self-respect). *Sustains* change. MET's job is to convert extrinsic pressure into intrinsic reasons.

Change talk vs sustain talk

Change talk = the patient's own statements favouring change. **Sustain talk** *(statements defending the status quo — keeping things as they are)* = statements favouring continued use. The frequency and strength of change talk predicts behaviour change; MET selectively evokes and reinforces it while never arguing against sustain talk.

Discrepancy

The felt **gap between current behaviour and important goals/values**. MET *develops* this gap — but from the patient's own perspective, never by lecturing. The patient, not the therapist, should voice the discrepancy.

Self-efficacy & personal responsibility

The responsibility and the choice to change remain with the patient throughout. This both respects autonomy and, paradoxically, increases the likelihood of change.

7 · The MI Spirit & Four Processes · 8 · Why Confrontation Backfires

The MI spirit — PACE (the attitude behind every technique)

Element	Meaning
P — Partnership	Work *with* the patient as collaborators, not *on* them as an expert fixing a problem.
A — Acceptance	Absolute worth, accurate empathy, autonomy support, and affirmation of strengths.
C — Compassion	Actively prioritise the patient's welfare, not the clinician's or institution's agenda.
E — Evocation	Draw motivation *out of* the patient (“you already have what you need”), rather than install it.

The four processes of MI/MET

These flow in order but you can loop back any time:

1. **Engaging** — build a trusting, collaborative relationship.
2. **Focusing** — agree a direction/goal for the work.
3. **Evoking** — elicit the patient's own motivations (change talk). *This is the heart of MET.*
4. **Planning** — develop commitment and a concrete change plan.

Why confrontation, arguing, blaming & lecturing backfire

The **“righting reflex”** *(the helper's natural urge to fix things and set the person straight)* is the most common clinical trap. When a clinician argues for change, **psychological reactance** leads the patient to argue the other side — effectively talking themselves *into* continued use. Labelling (“you're an addict in denial”) provokes shame and defence. In MET, if the patient is defending use, the **therapist changes course** — reflect, roll with it, re-affirm autonomy.

> **The MET maxim:** arguments for change should come from the *patient's* mouth, not the therapist's.

9 · The Complete Technique Compendium

Every technique MET/MI uses, grouped by purpose. For each: **what it is · why/when to use it · how to do it · an example.** Book Two shows which technique belongs in which session.

Group A · OARS — the foundational micro-skills

Open questions A1

What: questions that cannot be answered yes/no and invite elaboration. **Why:** open topics, hand the floor to the patient, gather change talk. **How:** start with “what”, “how”, “tell me about…”; avoid stacking questions. Aim for a high ratio of reflections to questions.

> **T:** “What's a day like for you when you don't use?”

Affirmations A2

What: genuine recognition of the patient's strengths, efforts or intentions — *not* praise or flattery. **Why:** builds self-efficacy and the alliance; counters shame. **How:** comment on a specific strength or effort you have actually observed; frame around the patient (“you…”), not yourself (“I'm proud of you”).

> **T:** “You came in today even though it's uncomfortable — that takes real resolve.”

Reflective listening A3

What: saying back the *meaning* you heard, as a statement (not a question). **Why:** conveys empathy, checks understanding, and *selectively* steers by choosing what to reflect. **How:** make a reasonable guess at meaning; let your voice drop at the end so it lands as a statement.

> **T:** “You're worried this is slipping out of your control.”

Summaries A4

What: pulling several threads together. Three kinds: **collecting** (gather change talk), **linking** (connect current to earlier statements), **transitional** (shift to a new phase). **Why:** demonstrates listening, concentrates change talk, bridges to planning. **How:** end with a "key question".

> **T:** "So money, your health and your daughter all pull you toward change, while the one pull back is how it helps you relax. Where does that leave you?"

Group B · Reflection techniques (the skill most used in MET)

Simple reflection B1

What/How: repeat or lightly rephrase content; keeps conversation moving, low risk. **Example:** P: "I'm tired of it." T: "You've had enough."

Complex reflection B2

What/How: add meaning, feeling, or "continue the paragraph" — a deeper guess. **Example:** P: "I'm tired of it." T: "Part of you is ready for something different."

Double-sided reflection B3

What/How: hold both sides of ambivalence in one sentence, ending on the change side (recency effect). **Example:** "You enjoy how it relaxes you, *and* you're worried about your health."

Amplified reflection B4

What/How: reflect sustain talk slightly over-stated so the patient self-corrects — use sparingly & without sarcasm. **Example:** P: "It's not that bad." T: "So there's really no downside at all." P: "Well, the debt *is* bad..."

Reflection of feeling B5

What/How: name the emotion to deepen empathy. **Example:** "That really frightened you."

9 · Techniques — Group C · Evoking Change Talk

Evoking is the engine of MET. These techniques pull the patient's own motivational language into the room.

Evocative questions (DARN) C1

What: directly ask for Desire, Ability, Reasons, Need. **How:** “Why might you want to change? How could you, if you decided? What are the best reasons? How important is it?” **Example:** “If you did decide to cut down, how might you go about it?”

Importance & confidence rulers C2

What: a 0–10 scale for how important change is, and how confident the patient feels. **How:** ask the number, then **“Why that number and not a lower one?”** (pulls change talk) and “What would move you one point higher?” Never ask “why not higher?” (pulls sustain talk). **Example:** “You said importance is 6 — why a 6 and not a 3?”

Querying extremes C3

What/How: ask about the best and worst possible outcomes to surface motivation. **Example:** “What's the worst that could happen if nothing changes? And the best if it did?”

Looking back / looking forward C4

What/How: contrast life before the problem, or imagine a changed future. **Example:** “How were things before all this? Where would you like to be a year from now?”

Exploring goals & values C5

What/How: elicit core values, then explore how use fits — develops discrepancy. **Example:** “Being a good father is everything to you — how does using fit with that?”

Elaborating / asking for examples C6

What/How: when change talk appears, deepen it. **Example:** “Tell me more about that.” / “When was the last time that happened?”

The key question C7

What/How: an open question that opens the transition to planning, asked after a good summary. **Example:** “So, where does this leave you?”

Affirming & summarising change talk (the “bouquet”) C8

What/How: gather the patient's change statements and hand them back. **Example:** “You've named three strong reasons — your health, your marriage, and the money.”

9 · Techniques — Group D · Sustain Talk & Discord · Group E · Information

Discord *(tension/strain in the relationship — distinct from the patient's own ambivalence)* and sustain talk are signals to change *your* approach, not to push harder.

Rolling with resistance D1

What/How: don't oppose; move alongside and let momentum shift. Reflect instead of counter-arguing. **Example:** P: “I'm not quitting.” T: “Quitting isn't on the table for you right now.”

Emphasising personal autonomy D2

What/How: explicitly affirm the patient's right to choose — defuses reactance. **Example:** “It's completely your call — nobody can decide this for you.”

Reframing D3

What/How: offer a new, more useful meaning for the same facts. **Example:** “Your family's nagging — maybe that's a sign of how much they want you back.”

Agreement with a twist D4

What/How: reflect/agree, then gently add a new angle. **Example:** “You're right it's your decision — and you're clearly weighing it carefully.”

Shifting focus D5

What/How: move away from a stuck point to reduce friction. **Example:** “Let's set the big decision aside and just look at this week.”

Coming alongside D6

What/How: briefly side with the status quo to defuse a stand-off. **Example:** “Maybe using matters so much right now that giving it up isn't worth it.”

Repair (apologise / affirm) in discord D7

What/How: when tension rises, mend the alliance. **Example:** “I've pushed too hard — I'm sorry. This is your call.”

Group E · Information, feedback & advice

Elicit–Provide–Elicit (E-P-E) E1

What: the MET way to give information or advice without lecturing. **How: Elicit** permission & what the patient already knows **Provide** focused, neutral information **Elicit** their reaction. **Example:** “What do you know about the medicine for cravings? … May I add a little? … What do you make of that?”

Personalised assessment feedback E2

What: the defining MET “enhancement” — present the patient's assessment results against norms, neutrally, one finding at a time. **Example:** “Your reported use is in the high-risk range compared with most men your age. How does that land?”

Ask permission E3

What/How: always precede advice/info with a request — protects autonomy. **Example:** “Would it be okay if I shared something I've seen help others?”

Menu of options E4

What/How: offer choices rather than a single prescription. **Example:** “Some people try X, others Y or Z — what fits you best?”

9 · Techniques — Group F · Planning & Relapse-Prevention · Group G · Adolescents

Decisional balance F1

What/How: explicitly weigh pros & cons of using vs changing (best in contemplation). Honour the real benefits of use first. **Example:** four-quadrant exercise (see Book Two worksheet 7).

Change-plan negotiation F2

What/How: co-author (not prescribe) goal, reasons, steps, supports, obstacles. The patient's words carry the commitment.

SMART goal-setting F3

What/How: make steps Specific, Measurable, Achievable, Relevant, Time-bound. Underpins the short-term & long-term goal worksheets in Book Two.

Functional analysis of triggers F4

What/How: map antecedents use consequences to reveal patterns. **Example:** "What's usually happening right before you use?"

"If-then" coping plans (implementation intentions) F5

What/How: pre-decide a response to each high-risk situation. **Example:** "*If* I'm offered at a function, *then* I step out and call my brother."

Urge-surfing F6

What/How: observe a craving rise, peak and fall like a wave without acting on it; it passes.

"Play the tape forward" F7

What/How: imagine the full sequence and consequences of using before acting. **Example:** "Picture how tonight actually ends if you use."

Refusal-skills rehearsal / role-play F8

What/How: practise saying no and leaving situations; therapist plays the offering friend.

Lapse-vs-relapse reframing F9

What/How: prevent the **abstinence-violation effect** *(the all-or-nothing reaction that turns one slip into a full relapse)*. **Example:** “One slip doesn't erase your progress.”

Supporting self-efficacy F10

What/How: recall past successes, affirm small wins, break goals down. **Example:** “You've done hard things before.”

Social-support & family involvement F11

What/How: recruit helpful (non-policing) support with the patient's consent.

Pharmacotherapy coordination F12

What/How: link MET with MOUD / NRT / naltrexone via the treating doctor; MET complements medication.

Booster sessions F13

What/How: brief repeat MET cycles at 1/3/6/12 months — review, re-rate rulers, re-plan.

Group G · Adolescent-specific adaptations ADOLESCENT

- **Autonomy first & loudest** — teens are developmentally primed to resist authority; avoid the parent/expert stance.
- **Present-focused evoking** — use money, freedom, image, sport, school rather than distant health risks.
- **Longer engagement, shorter sessions**, the young person's own language, comfort with silence.
- **Confidentiality balanced with safeguarding** and age-appropriate family involvement.

Techniques drawn from Miller & Rollnick (MI 3e), the Project MATCH MET Manual, Naar-King & Suarez, and relapse-prevention literature (Marlatt & Gordon).

10 · Change Talk in Depth — DARN-CAT

Change talk comes in two layers. **Preparatory** change talk (DARN) signals the person is *considering* change; **mobilising** change talk (CAT) signals they are *moving*. Mobilising talk best predicts action.

Code	Layer	Meaning	Substance-use example
D — Desire	Preparatory	want, wish, would like	"I **want** to be clean for my kids."
A — Ability	Preparatory	can, could, able to	"I **could** probably cut the evening dose."
R — Reasons	Preparatory	specific if/then arguments	"If I stop smoking, my chest will clear up."
N — Need	Preparatory	must, have to, need to	"I **have to** do something — the debt is out of hand."
C — Commitment	Mobilising	will, going to, promise, intend	"I'**m going to** see the doctor on Monday."
A — Activation	Mobilising	ready, willing, prepared	"I'**m ready** to give it a real try."
T — Taking steps	Mobilising	already acting	"I **threw out** my stash yesterday."

Recognising & responding to sustain talk — without confrontation

P: "Smoking is the only thing that calms my nerves."
T (reflect, don't challenge): "Right now it's your main way to cope with stress." **P:** "Yeah." **T (evoke):** "If you ever did decide to change, what else has helped you relax, even a little?" **patient supplies alternatives (change talk).**

> **Core move for sustain talk: reflect emphasise autonomy ask an evocative question.** Never "but you should…".

Rewriting unhelpful statements into MET-consistent ones

Unhelpful (confrontational / righting reflex)	MET-consistent rewrite
“You're in denial about being an addict.”	“You see this differently from others. Tell me how *you* see it.”
“You have to quit, period.”	“What you do is your call. What are you thinking might work for you?”
“Keep this up and you'll end up dead.”	“Can I share one health finding? … What do you make of it?”
“Why do you keep making excuses?”	“There are real reasons it's hard to stop. What are the biggest ones?”
“That's just the addiction talking.”	“Part of you wants to keep using and part doesn't — that's the tug-of-war.”
“You failed last time because you weren't serious.”	“Last time you managed three weeks. What worked, even briefly?”
“You need rehab now.”	“There are a few options — shall we walk through them together?”
“Stop lying about how much you use.”	“Exact amounts are hard to pin down. Roughly, on a heavy day?”

11 · Evidence, Limitations & Ethics

Evidence (summary)

MET and MI have a substantial evidence base in substance-use treatment. In **Project MATCH**, 4-session MET produced drinking outcomes broadly comparable to longer CBT and Twelve-Step Facilitation — notable given fewer sessions. Reviews and the **Cochrane** literature on MI/MET report benefits for engagement, treatment retention *(staying in treatment)* and reduced use, though effect sizes vary by substance, population, setting and comparison condition, and some effects are modest or short-lived without follow-up. Prioritise the original Project MATCH reports, Miller & Rollnick, Naar-King & Suarez, and Cochrane reviews as verifiable sources; figures here are summarised, not exact statistics.

> **MET can:** improve engagement/retention, strengthen motivation and commitment, reduce use for many, and act as a low-intensity entry point. **MET cannot:** replace detox, pharmacotherapy or management of severe dependence; guarantee abstinence; or "cure" addiction through motivation alone.

Limitations

- Outcomes vary across substances (stronger engagement evidence for alcohol/cannabis than for some stimulant outcomes) and populations.
- Effects depend heavily on treatment context and **continuing care/follow-up**.
- Motivation is necessary but not sufficient — pair with medical treatment and skills where indicated.

Ethical practice

- **Informed consent & confidentiality** — explain purpose, voluntariness and limits (risk to self/others, legal mandates; for minors, clarify with the young person and caregivers).
- **Respect for autonomy** — the patient chooses goals; avoid coercion and confrontation.
- **Non-stigmatising language** — "a person who uses heroin", not "addict/junkie"; describe behaviour, not identity.
- **Cultural sensitivity** — adapt to family structure, religion, gender and local norms.
- **Appropriate referral & multidisciplinary care** — coordinate with physicians, psychiatry, social work and community resources.
- Do **not overstate efficacy** or claim motivation alone resolves dependence.

12 · Glossary of Difficult Terms

- **Abstinence** — stopping use completely.
- **Abstinence-violation effect** — all-or-nothing reaction turning one slip into a full relapse.
- **Adherence** — sticking to agreed treatment.
- **Ambivalence** — mixed feelings; wanting and not wanting change at once.
- **Autonomy** — a person's right to make their own choices.
- **Cognitive dissonance** — discomfort of holding two conflicting ideas.
- **Cue reactivity** — how strongly reminders trigger urges.
- **Decisional balance** — weighing pros & cons of using vs changing.
- **Delirium tremens (DTs)** — severe, dangerous alcohol-withdrawal state.
- **Detoxification (detox)** — medically-supervised withdrawal.
- **Discord** — tension/strain in the therapeutic relationship.
- **Discrepancy** — gap between current behaviour and goals/values.
- **Empathy** — understanding and reflecting another's feelings.
- **Evocation** — drawing motivation out of the patient.
- **Extrinsic / Intrinsic motivation** — from outside pressure / from within.
- **Euphoria** — intense pleasure or “high”.
- **MOUD** — medications for opioid use disorder (e.g., buprenorphine, methadone).
- **Naloxone** — antidote that reverses opioid overdose.
- **NRT** — nicotine replacement therapy (patch, gum, etc.).
- **OCR** — optical character recognition (scanned images text).
- **Overdose** — a dangerous excess dose.
- **Pharmacotherapy** — treatment with medicines.
- **Psychological reactance** — pushing back when freedom feels threatened.
- **Relapse / lapse** — fuller return to use / a single slip.
- **Retention** — staying in treatment.
- **Righting reflex** — the helper's urge to fix/correct the person.
- **Self-efficacy** — belief in one's own ability to succeed.
- **Sustain talk** — the patient's words defending the status quo.
- **Tolerance** — body needing more drug for the same effect.
- **Urge-surfing** — riding out a craving until it passes.

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M.Phil. Clinical Psychology · MET Book One: Theory & Techniques

Continue to **Book Two — Session-by-Session Guide & Worksheet Pack** for applying these techniques session by session.